



MADISON COUNTY MEMORIAL HOSP.
224 NW CRANE AVE, MADISON, FL 32340 - 850-973-2271

Patient Name: MICHEAL DOWDY

DOB: 08/17/1964

MRN: 32871

Account #: 1072721

Note Type: ED Physician Note

All Demographics									
Patient Name	DOB	Age	Sex	Visit Number	Admission Date/Time	Attending Physician	Date of Service	Room and Bed	Emergency Contact
DOWDY, MICHEAL	08/17/1964	61 years	Male	1072721	07/01/2026 08:57	DANIEL KUDER	07/01/2026	ED-T1	

07/01/2026 10:17

History of Present Illness

Chief Complaint: **Chief Complaint:** FINGER LAC

HPI:

61-year-old male presents with laceration to the right middle finger distal phalanx on a skill saw just prior to arrival.

Denies weakness or decreased sensation distally or any other associated injuries.

Past Medical/Surgical/Family/Social History

Past Medical History:

All Problems						
Problem	ICD Code	Onset Date	Resolved Date	Age	Status	Comment
Chronic depression (disorder)	F34.1				Active	
Gastroesophageal reflux disease (disorder)	K21.9				Active	
Laceration without foreign body of right middle finger without damage to nail, initial encounter	S61.212A				Active	

Past Surgical History:

Surgery List

Total replacement of right knee joint,



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Gastric bypass,
Chole,

Past Family History:

Family History List: No Family History Available

Social History:

Smoking Status: Never smoker, **Cessation Education:**

Allergy List

Sulfa Antibiotics, Medication

Home Meds: Dose and Freq		
Medication	Dosage	Frequency
CeleBREX 200 MG Oral Capsule	200 MG	
Lexapro 20MG Oral Tablet		DAILY
Protonix 40 MG Oral Tablet, Delayed Release		DAILY
Singulair 10 MG Oral Tablet	10 MG	

Review of Systems

ROS Negative Except as Noted in HPI

X	Yes
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Physical Exam

Vital Signs: This Visit																							
Date/Time	BP (mm/Hg)	BP Position/ Site	MAP (mm/Hg)	Heart Rate	Pulse Site	Resp	Temp (°C)	Temp (°F)	SPO2%	O2 L/min	FiO2	EtCO2 (mm/Hg)	O2 Device	Blood Sugar	Pain Score	Height (cm)	Height (in)	Weight (kg)	Weight (lbs/ozs)	Scale	BMI	BSA	Head Cir (cm)
07/01/2026 09:14	138/91	Sitting/ Left Arm	107	79	Pulse Ox	16	36.9 Oral	98.5 Oral	98 %				Room Air 21%		10	177.8 cm	70 in	94.8 kg	209.0		29.99	2.16	

GENERAL: Well appearing and well nourished. In no apparent distress.

HEAD: Normocephalic, atraumatic.

EYES: Pupils are equal, round and reactive to light. Extraocular movement intact. Without injection or icterus.

EARS: Normal external exam without injury

MOUTH/THROAT: Airway is patent. Moist mucous membranes.



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NECK: Supple, without masses. Trachea midline

CHEST/LUNGS: Full and equal chest rise and fall. Unlabored respirations.

Cardiovascular: Strong and equal pulses b/l. No lower extremity edema.

MUSCULOSKELETAL: Normal ROM, No deformity. Normal tendon function to the right middle finger

SKIN: No rashes. Warm without jaundice or cyanosis. 2 cm laceration transverse to the distal phalanx of the right middle finger extensor aspect.

NEUROLOGIC: GCS 15, No focal weakness. Normal gait

PSYCHIATRIC: Mood and affect are appropriate for age. No SI

ED Course

10:20

Laceration Repair

Indication: Laceration to right middle finger

Laceration length: 2 cm

Linear laceraton

Procedure

Verbal consent obtained

Wound prepped with hibiclens and draped.

Anesthesia with bupivacaine 0.25%/ Lidocaine 1% without epi

Wound irrigated with NS pressure wash

Wound explored through FROM with no foreign bodies or injury to deeper structures noted

Wound closed with Ethilon 4-0 #3 simple interrupted technique

Antibiotic ointment applied and return precautions discussed with patient who will return here for suture removal

Pt tolerated procedure well.



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Note Type: ED Physician Note

Abnormal Results: Last 24 Hours: No Labs Available

Lab Results: Last 24 Hours: No Labs Available

Ordered Meds List: No Current Medications Available

Radiology:

Preliminary radiology interpretation based on my review pending official read:

X-ray right hand three-view-

No fracture, dislocation, or foreign body

Review of Diagnostics

I have personally reviewed and interpreted the results of the diagnostic findings as ordered, including the need for these tests based upon the history and physical exam performed to assure we rule out any emergent/urgent findings.

This also includes my independent review and analysis of any EKG or radiographic study as ordered, as timely preliminary interpretation is important to the care of the patient. Any pertinent findings will be addressed as needed, as well as relayed to the patient or appropriate party so they are aware.

MDM

patient presents with complicated injury to the right middle finger from skill saw. Concern for fracture, foreign body, neurovascular injury, and others The patient is well appearing, and nontoxic. After careful history, physical exam, vital signs, and review of any laboratory or diagnostic data as ordered, I made the decision the patient does not appear to have a life threatening illness and can be carefully discharged with close follow-up. I do not feel any further imaging or laboratory data is needed at this time. I have considered both emergent, urgent disease pathology, and the current DIFFERENTIAL DIAGNOSIS including but not limited to: those listed above

Fracture

N/V injury

tendon injury

Foreign body

Chronic conditions addressed impacting care:



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Note Type: ED Physician Note

I have given written and verbal discharge instructions.

Patient advised of the importance of following up with their PCP concerning findings, treatment, and status post treatment. Extensive conversation with patient regarding findings, differential diagnosis, return precautions and treatment plan. Patient was educated on any prescriptions and proper usage as well as the importance of following up with primary care in a timely fashion for ongoing health and wellness management. The nursing staff has reiterated the disposition plan as well.

Diagnosis Today

Problem: Laceration without foreign body of right middle finger without damage to nail, initial encounter, ICD

Code: S61.212A, **Status:** Active, **Addressed Date:** 07/01/2026

Plan

keep wound clean and bandaged with antibiotic ointment until healed

Return in 7-10 days for suture removal.

Take Tylenol over the counter as directed for pain

Apply ice pack to reduce swelling and pain

Return immediately for any unexpected change in your condition

Thank you for choosing our Emergency Department for your healthcare! Please take your medicines prescribed as directed and be assured that you follow up with the physician provided or your PCP in the next 1-2 days to assure you are improving. All medical problems cannot be reasonably diagnosed in your ED visit today. Return for any changes or concerns, including if your condition does not improve or you are unable to obtain follow-up. Some final results, including radiology and laboratory reports, do not return the same day, but are available on the patient portal or can be obtained through your PCP.

Medications given this visit:

Ordered & Completed Meds Table					
Ordered Medication	Start Date/Time	Dosage	Route	Frequency	Status
VAC- TETANUS DIPH TOX ADULT INJ	07/01/2026	1 DOSE	IM OPTIONS	X1	completed

new Medications:



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Note Type: ED Physician Note

Today							
Medication	Dosage	Route	Frequency	Start Date	Stop Date	Prescribing MD	Special Instructions
Ibuprofen 600 MG Oral Tablet	1 TABLET	ORAL	AS NEEDED EVERY 8 HOURS	07/01/2026		KUDER DANIEL	TAKE 1 TABLET ORAL AS NEEDED EVERY 8 HOURS FOR PAIN

Discharge Medications

Ibuprofen 600 MG Oral Tablet



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Note Type: ED Nurse Triage

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DOWDY, MICHEAL	08/17/1964	61 years	Male	1072721	07/01/2026 08:57	DANIEL KUDER	07/01/2026	ED-T1	

07/01/2026 09:43

Triage

Mode of Arrival: POV

Triage Location: ED TRA 2

Triage Acuity / Emergency Severity Index: 3

Presenting Problem(s) / Complaint(s): FINGER LACERATION

Onset of Symptoms / Time of Injury: 07/01/2026 0830

Therapies Initiated Prior to Arrival: CLEANSED

Procedures / Tests Ordered / Performed at Triage: NONE

Pain Assessment		
Pain Scale	Location	Quality
10	RIGHT HAND 3RD DIGIT	THROBBING

Vaccines							
Influenza		Pneumonia			Tetanus		Childhood Immunizations
	Current for this season		Current / Within 5 years			Current	X Up to date
X	Not current	X	Not current	Not applicable	X	Not current	Not up to date

Vital Signs: This Visit																							
Date/Time	BP (mm/Hg)	BP Position/ Site	MAP (mm/Hg)	Heart Rate	Pulse Site	Resp	Temp (°C)	Temp (°F)	SPO2%	O2 L/min	FiO2	EtCO2 (mm/Hg)	O2 Device	Blood Sugar	Pain Score	Height (cm)	Height (in)	Weight (kg)	Weight (lbs/ ozs)	Scale	BMI	BSA	Head Cir (cm)
07/01/2026 09:14	138/91	Sitting/ Left Arm	107	79	Pulse Ox	16	36.9 Oral	98.5 Oral	98 %				Room Air 21%		10	177.8 cm	70 in	94.8 kg	209.0		29.99	2.16	

Allergy Table		
Allergen	Type	Reaction
Sulfa Antibiotics	Medication	



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Note Type: ED Nurse Triage

Active Home Meds						
Medication	Dosage	Route	Frequency	Last Dose Date	Prescribing MD	Special Instructions
CeleBREX 200 MG Oral Capsule	200 MG	ORAL				
Lexapro 20MG Oral Tablet		by mouth	DAILY			
Protonix 40 MG Oral Tablet, Delayed Release		by mouth	DAILY			
Singulair 10 MG Oral Tablet	10 MG	ORAL				

Narrative Notes
61 YO MALE PRESENTS VIA POV WITH CC OF LACERATION TO THE RIGHT HAND 3RD DIGIT. PT AMBULATED TO ED ROOM ED TRAUMA 2 WHERE VITALS WERE RECORDED. PT REPORTS THAT THEIR SYMPTOMS STARTED 0830. PT RATES THEIR PAIN 10/10 AND DESCRIBES IT AS THROBBING. / DENIES PRE-HOSPITAL CARE. PT IS A&OX4 AND APPEARS TO BE IN NAD. PT WAS PLACED ON THE MONITORS AND VS WERE OBTAINED. PROVIDER NOTIFIED. NO ACTIVE BLEEDING NOTED AT THIS TIME

ED Laceration

Location: RIGHT HAND 3RD DIGIT

Length: 2

Depth/shape/contamination: Linear Clean Into subcutaneous Flap



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Note Type: ED Nurse Triage



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Note Type: ED Nurses Note

consent obtained and TD given in the right deltoid, patient tolerated well



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Note Type: ED Nurses Note

07/01/2026 09:19

Provider at bedside at this time.



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Note Type: ED Nurses Note

07/01/2026 09:21

Radiology at bedside for x-ray at this time.



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Note Type: ED Nurses Note

NON ADHERENT DRESSING APPLIED WRAPPED WITH KERLEX AND COBAN



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Note Type: ED Nurse Disposition Note

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07/01/2026 10:35

DISPOSITION

Disposition Checklist	
Stop times completed in order chronology for all IV fluids / IVPB medications	I&O's entered in vital signs

DISCHARGE

Discharge To: HOME

Discharge Instructions Given to: PT

Condition: GOOD

Verbalizes understanding of instructions given: YES

Prescriptions Given to: PHARMACY

Time of Departure: 1035

Mode of Departure: AMBULATORY OUT OF ED

Accompanied by: PT SELF



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Note Type: ED Discharge Instructions

All Demographics									
Patient Name	DOB	Age	Sex	Visit Number	Admission Date/Time	Attending Physician	Date of Service	Room and Bed	Emergency Contact
DOWDY, MICHEAL	08/17/1964	61 years	Male	1072721	07/01/2026 08:57	DANIEL KUDER	07/01/2026	ED-T1	

07/01/2026 10:18

Chief Complaint: FINGER LAC

Diagnosis:

Diagnosis Today				
Problem	ICD Code	Status	Comment	Addressed Date
Laceration without foreign body of right middle finger without damage to nail, initial encounter	S61.212A	Active		07/01/2026

Diet: As Tolerated

Activity: Activity as tolerated

Additional Instructions: keep wound clean and bandaged with antibiotic ointment until healed

Return in 7-10 days for suture removal.

Take Tylenol over the counter as directed for pain

Apply ice pack to reduce swelling and pain

Return immediately for any unexpected change in your condition

Thank you for choosing our Emergency Department for your healthcare! Please take your medicines prescribed as directed and be assured that you follow up with the physician provided or your PCP in the next 1-2 days to assure you are improving. All medical problems cannot be reasonably diagnosed in your ED visit today. Return for any changes or concerns, including if your condition does not improve or you are unable to obtain follow-up. Some final results, including radiology and laboratory reports, do not return the same day, but are available on the patient portal or can be obtained through your PCP.

Medications administered during this visit:



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Note Type: ED Discharge Instructions

Ordered & Completed Meds Table					
Ordered Medication	Start Date/Time	Dosage	Route	Frequency	Status
VAC- TETANUS DIPH TOX ADULT INJ	07/01/2026	1 DOSE	IM OPTIONS	X1	completed

Follow-up with: Your primary care provider in 1-3 days.

New Scripts:

Discharge Medications

Ibuprofen 600 MG Oral Tablet

Follow-up with your Healthcare Provider as directed.

Return to the Emergency Department if symptoms persist or worsen.

Patient / Guardian

Date

Nurse

Date